2.10 Dehydration and Hypovolemia*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel thirsty and dizzy." **Objective** • BP 95/60 mmHg, HR 115/min • Urine 20 mL/hour, dark • Reduced skin turgor	**ACTUAL** Inadequate fluid volume related to active fluid loss as evidenced by hypotension, tachycardia and low urine output.	• Within 8 hours, urine output of at least 0.5 mL/kg/hour and stable vital signs.	• Record vital signs hourly until stable. • Give prescribed IV fluids and check the rate regularly. • Measure urine output hourly and keep an intake and output chart. • Weigh daily at the same time. • Follow electrolytes, urea and creatinine. • Encourage oral fluids once allowed.	• Goal reached when BP, HR and urine output normalize. • A poor response leads to medical review of the fluid plan.
Subjective • "My mouth is very dry." **Objective** • Lips dry and cracked • Dry tongue	**ACTUAL** Impaired oral mucous membrane integrity related to dehydration as evidenced by dry, cracked lips and tongue.	• Moist oral mucosa within 24 hours.	• Provide mouth care every 4 hours. • Apply lip moisturizer. • Offer frequent small sips if permitted.	• Met when the mouth is moist.
Subjective • Still vomiting **Objective** • Ongoing fluid loss • BP trending down	**RISK** Risk for shock as evidenced by risk factors: hypovolemia.	• MAP stays above 65 mmHg and consciousness remains normal.	• Assess level of consciousness and capillary refill. • Report a systolic BP below 90 mmHg. • Keep a large-bore cannula in place. • Prepare for fluid resuscitation as ordered.	• Achieved if no signs of shock develop.

* **References for this care plan:** [17], [35], [37], [64], [108]. Full entries are listed in the References section of the CPMN book.